UNDERSTANDING DISABILITY

- The term illness is far from obvious…the same is true for the term **disability**
- Medical model of disability…deficit that should be cured (isn't it that an individual ***has***?)
- According to PWDs the answer is no…how others respond to those **differences** and the choices others have made in constructing the **social and physical environment**

- This reflects a **sociological model of** disability… (*Individualizing disability*…social attitudes & environment rather than rehabilitating)
- Not just two, there are several models of disability:

The moral and/or religious model

The medical model

The social model

The identity model

The human rights model

The charity model

The economic model

The limits model

https://www.youtube.com/watch?v=Jig5uNbN3xk&t=11s

Assistive Technology (AT) and Disability

- AT can be any technological/technical interventions/aid that help people with disabilities function within their environment
- These ATs
  - Assist them in learning
  - Make the environment more accessible
  - Enable them to compete in the workplace
  - Enhance their independence
  - improve their overall quality of life
- However, the **societal meaning of disability** is changing due to introduction of technological interventions…(e.g. telephones and emails have given different meaning towards disability like deafness and blindness)

Technology and Childhood Disability

Impact of preventive technology

- Sometimes injuries result in disabling conditions in childhood… technological interventions prove that injuries are highly preventable
- The physical environment can be improved through technical inputs…(housing, automobile travel, pedestrian, playground design) …leads to significant reductions in injury-related mortality and disability in children
- These technical improvements can benefit children depending on the public (based on general design enhancement) and private (child car seat, child-protective car window lock) intervention… though certain interventions are legal, social disparities define their actual use and consequent result
- Not just in case of childhood, but the role of such preventive technology is also evident even in prenatal and neonatal stages

- Technology, in the form of screening initiatives, identify genetic or other indicators of disability risk on prenatal stage
- Genetic screening of prospective parents…risk-associated genetic profile/conditions…may result in termination of pregnancy (which again has many ethical, moral and legal questions)
- However, sometimes technology is developing new prenatal interventions like *fetal* surgery…
- But, we need to understand that the use of prenatal diagnostic technology is also characterized by significant social disparities (complex medical procedures or delivery infrastructures, legal codes of termination)
- Newborn screening programs are designed to identify those conditions which are present but not clinically recognizable at neonatal stage…it can be dealt with early initiation of special diet

- However, although new testing technologies can screen such metabolic and genetic disorders, many of these conditions are still *poorly understood* or have *no effective treatment*
- Therefore, four factors are interrelated
  - The technical ability to identify risk
  - The challenges of making sense of this knowledge
  - Using them for an efficient and effective outcome
  - And lastly, the humane response

Impact of therapeutic technology

- In general, children with disabilities rely more on technical interventions (like medications, specialized medical and even educational services)
- Technology can be helpful in case of certain conditions like cerebral palsy…these are designed to enhance children's functional abilities as they suffer from cognitive and motor disorders… improving the quality of life and social participation